



Greeting Protocol:

- **Mandatory for the provider:**

- Interpreter's name.
- Language you interpret in.
- Interpreter's ID #.
- Everything is going to be interpreted.

*Hello, this is _____, your **Ukrainian** interpreter, ID XXXXX. **Everything in the session will be interpreted.** How can I help you?/ Is there anything I need to know about the session?/ [Collect missing data]*

- **Mandatory for the LEP:**

- Interpreter's name.
- Language being interpreted into.
- Everything will be kept confidential.
- Everything in the session will be interpreted.

Добрий день, мене звати ____, я ваш перекладач англійської мови. Усе під час цієї сесії буде перекладатися та залишатиметься конфіденційним.

Optional add-ons

"Speak in short sentences. "

"Address questions to each other not to you."

"Speak in a loud and clear tone, pausing often."

"Who else is participating in the session today and what are their roles? If multiple parties are present, please remember to speak one at a time, allowing time for interpretation."

"I may intervene to ask for clarification or repetition, especially if there are audibility and/or visual challenges."

"I may ask end users to re-position the device for optimal setup."

Closing Protocol:

- **Video Call:**

1. Offer additional assistance (*this is mandatory*)
2. Remind doctor to rate the call
3. Say goodbye to doctor
4. Say goodbye to the LEP

*(1) Is there anything else I can help you with? / Do you need help with something else?
(2) Please remember to rate the call after we hang up. Your feedback is valuable and appreciated.
(3) Thank you for calling. Have a great day!
(4) "Гарного вам дня, пане / пані."*

- Audio Call:

1. Offer additional assistance
(this is mandatory)
2. State Interpreter's ID #
3. Say goodbye to doctor
4. Say goodbye to the LEP

(1) "Is there anything else I can help you with? / Do you need help with something else?"
(2) **This is interpreter ID#**_____.
(3) Thank you for calling / Thank you for using our services. Have a great day!"
(4) "Гарного вам дня, пане / пані."

Intervention Protocols:

Transparency

- 3-Step Process:

If you ever need to intervene to ask for repetition, clarification, verification, etc.

1. Identify yourself as the interpreter. Speak first to the party that will be waiting for you to request for clarification, repetition, etc.
2. Identify yourself as the interpreter to the other party. Request clarification, repetition, etc.
3. Resume interpreting.

To opposite party
(in target language):

*"This is the interpreter speaking/
Excuse me, the interpreter needs
to clarify/a repetition/etc."*

To the required party:

*"Interpreter speaking, can you
please repeat/clarify....?"*

- 3rd Person Protocol:

If you ever need to intervene to ask for repetition, clarification, verification, etc.

- Intervene during the session to remind them to speak to each other.
- If they continue to speak to you and not to each other, continue to interpret in first person.

*"This is the interpreter, [provider], may I
please ask you to speak directly to your
patient so that we can ensure accuracy and
avoid confusion?"*

*"Пане / пані, я перекладач. Будь ласка,
звертайтеся безпосередньо до лікаря,
щоб уникнути двозначностей і
забезпечити точність повідомлення.."*

Exceptions to Third Person

- Very young children (7 or younger)
- Emergencies summarizing
- Anyone who is confused (dementia/substance abuse/post surgery)
- Mentally ill patients
- Trauma triggered by situations in the call

**Specify the reason for using 3rd person in the call notes.*

- "Say No" Model:

Use the "Say No Model" when there is an inappropriate request from the provider or patient.

1. Be gracious

2. Offer 2 or 3 options

3. Give a reason

"Could you please do a sight translation of this document? As you see it is short and not complicated."

"Thank you for trusting my language skills and I understand that you need the information on this document to be transmitted. As an interpreter I can help you read it out loud and I'll interpret as you do or maybe you can give me a summary of the document orally and I can interpret it as well. I'm stating this due to company policy"

"Interpreter you have been amazing...can I send you a gift?"

"Thank you for your appreciation of my service. Maybe you can donate this to the hospital, and they can make better use of it/ maybe you can call customer service and let them know about my performance. Your appreciation for my job and kind words are more than enough for me"

"Interpreter you have been great and I would like to have you again, what are your working hours?"

"Thank you for your appreciation of my service. All of our interpreters are qualified, and you can call the line at any time since we have service 24/7, my hours can be changed so it wouldn't be something certain, I do appreciate your words, and you can always leave feedback about my performance"

- Hold Protocol:

• 10 Minute Hold

We can hold up to **10 minutes** for a provider to return. Pause your video when there's no provider in the room. You can **reset the hold time** if the provider continues to come back and asks you to keep waiting. We can only hold for more than 10 minutes if it's an **emergency** (according to interpreter's judgement).

Don't forget to:

1. Let the provider know we can only wait up to **10 minutes**.
2. **Inform the LEP** of the hold
3. Turn the **camera off and mute** our audio
4. Make a notation on **Call Notes**
5. Report on **Discord**

To the provider:

"This is the interpreter; I need to let you know that I have a 10-minute hold policy. I'll be here with my camera off until you come back."

To the patient:

Пане / пані, з вами говорить перекладач. Я вимкну свою камеру з міркувань безпеки до повернення лікаря. Я слухатиму все, що буде сказано (за потреби).

- **Flexible Hold Time**

If a provider asks us to remain on hold for longer than 10 minutes, we can be flexible. This means:

- If they provide us with a specific amount of time, we will remain on hold for whatever that amount of time is.
- If they provide us with a non-specific/uncountable amount of time, we will remain on hold for **20 minutes**.
- If we encounter a life or death situation, we must remain on hold for **as long as needed**.

Remember that any and all of these cases must be noted in the **Call Notes** box and reported on **Discord**.

- **1 Minute Hold**

At the beginning of the call, if you get a blue or black screen: you have 60 seconds to try to get a response from the other party, you can check on participants whether someone is connected. *Remember that this protocol should be done within the first 60 seconds.*

"Hello, this is your Ukranian interpreter. Is there anybody on the line?."

"This is your Ukranian interpreter. I'm unable to hear anyone on the line. I will disconnect from the call. I apologize for the inconvenience. Goodbye."

- **Troubleshooting Protocol:**

If there are technical issues such as poor audio and/ or video quality, you should follow the troubleshooting protocol.

1. Ask the provider to move the device closer/change the positioning or turn your camera off and immediately turn it on and see if the problem is resolved.
2. If the problem is not resolved, keep the camera off and continue with the interpretation.
3. If these steps are not successful, ask the provider if you may connect them to another interpreter to see if this helps, and provide the Technical Support number (1-866-449-4428) in case issues continue.

"This is the interpreter, [provider], there are some technical issues. Allow me to troubleshoot"

"This is the interpreter, [provider], we are still experiencing some technical issues. May I connect you with another interpreter to see if this helps? I apologize for the inconvenience"

- Dial-Out Protocol:

If you are asked to place a call, make sure you obtain this information before you dial the number.

1. What is the **phone number** you need me to dial?
2. **Who** should I say **is calling**?
3. May I please have the **name of the person / patient we're calling**?
4. Should I **leave a message** in case we reach a voicemail box?
5. Is there a **callback number**? (*In case not stated before*)

- Withdrawing from long calls:

• Calls over 90 minutes

For any calls lasting **90 minutes or longer**, it is recommended that interpreters self-monitor their performance and make the decision to **transfer the call if needed**.

Signs that you need to transfer a long call include:

- a. Increased fatigue
- b. Performance concerns
- c. Emotional distress responses
- d. Need to use the restroom or drink water

To the provider:

"[Provider], as the interpreter, I think we've reached a point in the conversation to switch out interpreters. Please remain on the line and you'll be connected to one of my colleagues momentarily. Thank you."

To the patient:

Як перекладач, вважаю, що ми дійшли до моменту розмови, коли доцільно змінити перекладача. Будь ласка, залишайтеся на лінії — вас негайно з'єднають з одним із моїх колег. Дякую.

• Calls over 120 minutes

If a call reaches **over 120 minutes** in duration, you are expected to transfer the call so you can take a break. We understand, as the interpreter, you may be aware the call will conclude shortly after 120 minutes and may feel comfortable staying on the call until the call is completed. **Remember that it is important to always use your best professional judgment and customer service during these situations.**

Keep in mind:

- The **end of your shift alone is not a valid reason** to withdraw from a call and should not be stated as such to the callers.
- If you **don't feel tired** when reaching 120 minutes, **you can remain in the call**.
- Don't forget to make a **notation on the Call Notes** box.
- **Report on Discord** and keep your TL informed about the situation.

In **sensitive situations**, ask yourself if it would do more harm than good to swap out interpreters:

- If yes, consider **asking the provider for a time frame of when the session may be complete since you may be able to stay for a short period.**
- If no and it will be lengthy, **proceed with suggesting switching out interpreters.**

Always use your best customer service and wait for an appropriate time to intervene.

- **Shadowing** between languages:

When the Provider speaks Ukrainian or the patient starts speaking English, you can stay on the line shadowing **in case they need you.**

To the provider:

"This is the interpreter, I can tell you speak Ukrainian/your patient speaks some English. I will stay here shadowing in case you need me. I'll be paying attention and will jump in if there's any piece of information you deem very important."

To the patient:

З вами говорить перекладач. Оскільки лікар розмовляє українською мовою / ви можете спілкуватися англійською, я залишатимусь на лінії та спостерігатиму на випадок, якщо буде потрібен(потрібна). Я уважно стежитиму за розмовою та втручатимусь, якщо буде повідомлено важливу інформацію.

Keep in mind:

- Remain **alert** and **keep taking notes** as if you were interpreting.
- **Be ready** if at some point you are summoned.
- Pay attention in case there is a misunderstanding and **be proactive** if an intervention is needed.
- Make sure that all the parties are **aware of your presence** and each other's roles.
- **Be transparent** at all times.

- **Direct Access Line (DAL):**

Some hospitals and healthcare facilities provide **Direct Access Lines (DALs)** so LEP patients can dial, select a number for their language and be connected to the interpreter directly.

DAL calls will appear as standard audio calls. As the **Call Sheet** loads, pay special attention to the Account which will contain Direct Access Line or DAL.

DAL call in MVCC

Call Form	
ACCESS CODE	CALL SHEET
Call details	
Interaction ID *	Account *
IR-0006129513	Direct Access Line - Testing
Unit ID *	Non Billable Reasons *
+15672042680	None
Language *	Department *
Klingon	DAL - 614-264-3000

Keep in mind:

- In some of these calls there's no phone number registered and our LEP offers to provide a phone number for us to Dial Out. Then, you can take the phone number if the LEP provides one in order to perform the Dial Out and proceed from then on.
- Any and all of these cases must be noted in the Call Notes box and any notes we need to add to the call should be completed during the call while still connected to all parties.

- Dual Interpretation:

Hospitals will often request dual interpretations when the patient (typically a minor) uses ASL to communicate, while the parent speaks a language other than English.

Dual interpretations are often so that both parties are able to communicate in their native language.



1. The Client may call in the operator or one of the interpreters first. Whoever is connected first should add the other needed language teammate.
2. It is advised to put the provider on a brief hold while you connect your team, so you can brief them on the session and that you will be interpreting together.
3. The ASL interpreter will take the lead on the interpretation, helping to ensure effective call flow and pausing.

Keep in mind:

- If the provider primarily needs to interact with the deaf patient, you should allow the interaction to occur while taking notes before asking to pause.
- If you need to ask for clarification or repetition, please identify yourself as **"this is the (ukranian) interpreter"**.
- Always remember to interpret each other's interpretation.
- **Be transparent** about any interaction with each one of the parties.
- Make a note on the Call Notes box that this was a dual team interpretation. Don't forget to report it on Discord as well!

- Call Notes:

Always make sure to leave a note during your call even if nothing eventful happen.

Keep in mind:

- Information needs to be accurate and grammatically correct.
- Don't be afraid to ask for spelling.
- Names must be written appropriately, using capital letters/upper case letters for initials.

If, when collecting data, someone doesn't want to provide full information:

- If they provide initials, then we can type the initials in.
- If they still refuse, you must type **N/A** in capital/upper case letters.
- Leave a note on the **Call Notes** box about them refusing to provide information.